Children's and Family Recuperative Care Admission Criteria and Eligibility (under 18 years old)

Pre-Screen:
A nurse will visit the hospital to pre-screen the family and child. They will collaborate with the social worker and the discharge nurse to ensure that the discharge process and care plan remain consistent after the child leaves the hospital.

Family:
Recuperative care is a voluntary program, and background checks will be conducted on all adults and legal guardians accompanying the child. These checks will be performed through Megan's Law and local courts.

Housing:
Maximum of 3 people in a room.
There will be one to two adults per room with one recuperative care child.
If the child gets readmitted into the hospital the process is as follows:

1. The family will be able to remain in recuperative unless admission is more than 1 week then
2. The family may be placed in a short-term hotel property in the surrounding city.

List of Accepted Diagnoses:
Asthma Exacerbations
Closed fractures including upper and lower extremity fractures with limited mobility.
Constipation requiring maintenance regimen.
Diabetes Mellitus with strict guidance and education
Post-allergic reaction monitoring discharged from the hospital.
Pyelonephritis treated with oral antibiotics.
Deep cuts requiring stitches.
Minor burns
Attention-deficit/hyperactive disorder (ADHD)
Anxiety Disorder
Autism Spectrum Disorder
Depression- mild stabilized on medication
Oppositional Defiant Disorder
Post-Traumatic Stress Disorder (PTSD)
Obsessive-Compulsive Disorder (OCD)

Infection Control:
A negative MRSA swab must be documented prior to admission for patients coming from an inpatient hospitalization. A COVID-19 swab (rapid test accepted) must also show a negative result within the last 24 hours. Additionally, all infectious diseases should be resolved for at least 24 hours without the use of fever reducers such as acetaminophen or ibuprofen.

Illegible included and but not limited to:

Colds
Conjunctivitis (pink eye)- unless on ophthalmic drops for 24 hours
Hand, Foot, Mouth Disease,
Fifth Disease,
Roseola,
Chicken Pox unless all blisters have scabbed over about 5-7 days and rash has appeared.
Gastroenteritis (stomach flu),
Impetigo,
Respiratory Syncytial Virus (RSV),
Croup,
Strep Throat,
Influenza,
Whooping Cough,
Meningitis,
Diarrhea,
Fever,
Head Lice,
Measles,
Viral Bronchitis,
Conduct disorders,
Depression- moderate to severe
Bipolar Disorder
Oppositional Defiant Disorder
Explosive Disorders
Schizophrenia,
Eating disorders
Substance abuse.

Vital Signs:
A minimum of five recent vital signs must be submitted, all of which should be within normal limits. These include blood pressure, pulse rate, respiration rate, body temperature, and oxygen saturation. For children with a stabilized history of Type 1 Diabetes, five stable blood sugar readings should also be included. Additionally, provide weight history at the time of hospital arrival and discharge.

ADLS: PARENT IS RESPONSIBLE FOR ADL's
The child must be able to complete all activities of daily living (ADLs) independently, including personal hygiene, or with assistance from a legal guardian. Benches are available for bathing, and a seated shower is acceptable. The child should be able to gather their own bathing and showering supplies, transfer to the shower area, and perform bathing independently or with help from a legal guardian. Additionally, the child must be able to use the toilet independently and/or change briefs/diapers independently or receive assistance from a legal guardian.

Mobility:
Physical therapy (PT) and occupational therapy (OT) notes must be sent for review. If using durable medical equipment (DME) such as a wheelchair or walker, the child must be able to use it independently. The child should be able to ambulate at least 100 feet (rest breaks are acceptable) at modified independence if not dependent on a wheelchair. Wheelchair-dependent child must be able to transfer independently; if any transfers require assistance, this must be provided by a legal guardian.

Mental Health:
The child must be mentally stable while on medication, with no suicidal ideations or commanding auditory or visual hallucinations. Also, be cleared by a psychiatrist.

Cognition:
The child must be able to understand information and medical education, or this responsibility falls to a legal guardian. The client should be capable of self-administering medications; if not, a legal guardian must administer them. Recuperative care medical staff will only observe the administration of medications.

End of Life:
Hospice services must be arranged before admission to recuperative care. The patient, family, and other involved parties must be informed that recuperative care staff cannot administer medications; this will be the responsibility of the hospice staff.

Other Factors:
A child will be deemed ineligible if they are verbally or physically aggressive toward staff if parents are unable to manage outbursts.
A child with IV access may be accepted if they have established home health services to manage their IV status, and home health must administer IV medications as needed.